
ADHD.ME

findable.

nearby.

continuous.

A patient-facing finder that matches people to GPs who actually do ADHD assessment, and a practice console that fills the appointments it creates — with the lift measured against a randomised control.

the permission changed. the pathway didn't.

A GP is now allowed to carry ADHD assessment end to end. There is still no way for a person to find out which GP near them actually does it — and no way for a practice to show that it can.

6–12 mo

typical wait for an adult assessment appointment

\$1k–\$2k

common out-of-pocket cost of a private assessment

1m+

Australians living with ADHD (2023 Senate inquiry)

0

public ways to find which GP near you does this

the rule just changed.

DEC 2025

Queensland becomes the first state to let GPs diagnose adult ADHD and initiate treatment — with no mandatory additional training gate.

MAR 2026

NSW opens Stage 2 GP training toward independent diagnosis. South Australia, Western Australia and the ACT follow.

2023

The Senate inquiry's chief recommendation: a National ADHD Framework built on shared and collaborative models of care that widen the GP's role.

finding someone was the hard part.

MISSED AT SCHOOL

Went through school without the question ever being asked. Came to it as an adult, unprompted.

WANTED ONE GP

The stated priority was getting connected to a consistent GP who would not be judgemental.

HARD TO RAISE AT ALL

Reported that GPs are currently difficult to discuss mental health with in a non-judgemental way.

COULD NOT FIND WHO DOES IT

Reported extreme difficulty finding which GP does ADHD prescribing, without already knowing someone.

THE GOAL IS FUNCTION

The motivation was to stop underachieving and use their potential — not to collect a label.

First patient interview, 16 August 2026. One participant — a signal to test, not evidence of demand. De-identified findings only, recorded with the participant’s knowledge. The interview programme continues.

THE MARKET IS THE PEOPLE NOBODY FOUND

two in three are not being treated.

ESTIMATED AUSTRALIANS LIVING WITH ADHD — 1.5 MILLION

470,000

dispensed ADHD medication

~1,030,000

estimated, not receiving treatment

The market is not the people already diagnosed. It is the two in three who never got that far.

Every one of them is a person who, at some point, tried to work out who to ask — and stopped. That is the moment ADHD.ME is built for, and it is the moment a practice never sees.

Prevalence estimate: ~1.5 million Australians living with ADHD (2023 estimate; 2–6% of adults, 6–10% of children). Treatment: ~470,000 Australians dispensed ADHD medication in 2022–23 (AIHW). Dispensing is a lower bound on diagnosis — some diagnosed people are not medicated by choice — so the untreated share is the ceiling, not the count of undiagnosed.

say it in your words. book with one person.

A patient-facing finder and a practice-facing console on one engine, running end to end today. No clinical language on any patient surface — the compliance linter fails the build if it appears.

- ✓ Say what you need — in your own words, including how you want to be treated.
- ✓ See who is near you — by suburb, language, care area, and how that clinician works.
- ✓ Book once — assessment, baseline and follow-up with the same person.

ADHD assessment
*that takes
you seriously.*

Describe the GP you're looking for...



[Try a demo scenario >](#)

the practice earns more. the patient pays less.

WHAT THE GP EARNs

\$0

an empty mid-week slot

\$128.35

Item 44 — the prolonged consult ADHD assessment actually bills

≈\$29,500

a year, from one prolonged slot a day

WHAT THE PATIENT SAVES

\$1k–\$2k

private adult assessment, today

\$270–\$600

saved out-of-pocket on a GP-led pathway

\$9,600

our Standard tier — for the whole practice, per year

One GP, one prolonged slot a day, covers the whole practice fee three times over.

MBS rebates at 1 August 2026. ADHD assessment and review bill as Level C and D, not Level B. Annual figure: one prolonged consult per working day × 46 weeks — conservative, and before any private gap. Patient saving is the out-of-pocket difference on a GP-led pathway in the states that now permit one.

built and verified. pre-pilot, and saying so.

BUILT AND VERIFIED

- ✓ First patient interview complete — discovery under way
- ✓ Finder and practice console running end to end on a synthetic engine
- ✓ Verify gate on every commit — typecheck, tests, build, dependency audit
- ✓ WCAG 2.1 AA sweep, zero violations, across every public route

NOT YET, AND WE WILL SAY SO

No real patient data. No live messages. No pilot practice yet.

Each sits behind a named founder gate the build stops at rather than crosses.

The next gate to open is a design-partner practice — which is what we are asking Bond for.

three founders. one of them yours.



Vikram Ganeshalingam

CO-FOUNDER

Final-year MD candidate, Bond University

"What a person meets when they first look for help."



Dr Anubhav Saxena

CO-FOUNDER · MBBS, FRACGP

Practising GP · University of Sydney

"A documented baseline before anything starts, then follow-up on a schedule."



Stefan Thottunkal

CO-FOUNDER

NOURISH, Stanford Medicine · Health Systems Innovation Lab, Harvard T.H. Chan

"Physician-in-training and health-systems researcher."

what we're asking Transformer for.

- 📁 A Launchpad grant toward the pilot quarter — non-equity.
- 🛡️ Two mentors: clinical governance, and an Ahpra advertising review of the name.
- 🏢 Introductions to three Gold Coast general practices as design partners.
- 👤 Transformer Hub space and coaching through the pilot.

Queensland is the first state where a GP can carry this pathway. Bond is in Queensland.